

InStride Health Response to HHS RFI: Accelerating the Adoption and Use of AI in Clinical Care

About InStride Health

InStride Health is a multi-state pediatric mental health provider delivering specialty care for children, adolescents, and young adults with complex anxiety and obsessive-compulsive disorder. As a provider organization directly accountable for clinical outcomes, licensure, and patient safety, we engage with AI tools as clinical decision support and operational support, embedded in real-world care delivery.

Our comments reflect **direct experience implementing and governing AI-supported clinical and operational workflows** across fragmented state regulatory environments, particularly in high-risk mental health and pediatric settings, including work examined in a recent case study conducted in collaboration with The Wharton School's Health Care Management Department. The case study evaluated InStride's implementation of an AI scribe and documented measurable reductions in clinician documentation burden and improvements in clinician experience through a human-in-the-loop, clinically governed AI deployment model.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

InStride Response:

For multi-state healthcare organizations seeking to innovate responsibly, the most immediate barrier to AI innovation is **state-by-state regulatory fragmentation combined with the absence of clear federal baseline guidance**. Requirements governing consent, data use, and clinical accountability, particularly in mental health and pediatric care, vary significantly across states. Regulations in states such as Illinois impose stricter standards that often necessitate designing to the most restrictive interpretation or managing complex, state-specific implementations. Without a consistent federal framework for AI use in clinical care, this fragmentation significantly increases development cost, slows testing, and impedes responsible scaling.

A second major barrier is **uncertainty around accountability and liability when AI supports clinical workflows**. Existing clinical, regulatory, and legal frameworks were designed for human decision-making and have not yet been clearly adapted to AI-supported care. Many AI tools

influence clinical judgment without being regulated as medical devices, and vendor contracts typically place responsibility entirely on providers. In the absence of clear federal guidance on how accountability is assigned in these scenarios, organizations that prioritize patient safety must limit experimentation and narrowly scope AI use cases, regardless of technical readiness.

An additional and increasingly salient barrier to adoption that we've observed is **concern among clinicians about the energy intensity and environmental impact of AI technologies**. Clinicians are becoming more aware of the computational and energy demands associated with certain AI applications, and these concerns can meaningfully limit willingness to adopt or expand AI use in care delivery settings. In the absence of shared standards, transparency, or best practices regarding energy-efficient AI deployment, organizations are left to navigate these tradeoffs independently. HHS could play an important coordinating role in advancing guidance, shared learning, and innovation around lower-energy AI approaches or offsetting efficiency gains, helping ensure that responsible AI adoption aligns with broader sustainability goals in healthcare.

Collectively, these barriers reflect the need for clearer, more consistent federal frameworks that enable innovation while maintaining accountability, patient safety, environmental responsibility, and public trust.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

InStride Response:

HHS should prioritize regulatory and programmatic approaches that **promote responsible AI use in clinical care, where AI supports but does not replace human clinical judgment**. Clear federal guidance that distinguishes decision-support tools from autonomous clinical decision-making would reduce uncertainty and enable providers to innovate with confidence while maintaining patient safety.

Regulatory expectations should explicitly establish that AI-generated outputs are advisory, require human review, and do not shift accountability away from licensed professionals. This clarity is particularly important for multi-state providers, as consistent federal guidance would help mitigate fragmented state-level interpretations that currently complicate responsible AI design and deployment.

From a payment policy perspective, HHS should align incentives with **demonstrated improvements in care quality, safety, and access**, rather than narrowly rewarding efficiency or throughput. In this context, care quality should be defined by clinically meaningful outcomes

and care processes, such as symptom improvement measured through validated clinical scales, appropriate level-of-care matching, timely access to services, treatment engagement and retention, continuity of care, and reductions in safety events or avoidable escalation. Many AI applications that meaningfully reduce clinician burden or improve continuity of care lack clear reimbursement pathways, limiting investment in higher-value use cases. Payment models that reward outcomes rather than prescriptive processes would give providers flexibility to adopt AI tools that support high-quality care while maintaining appropriate human oversight. Outcome-aligned payment models would encourage innovation that benefits patients while discouraging premature deployment of unproven or overly expansive tools.

HHS could more effectively accelerate responsible adoption by expanding value-based and outcomes-oriented reimbursement models that allow providers to determine how best to deliver care. Such approaches would create durable incentives for providers to evaluate, adopt, and govern AI tools based on real-world clinical impact, while preserving accountability and public trust.

3. For non-medical devices, what novel legal and implementation issues exist and what role should HHS play to help address them?

InStride Response:

AI used in clinical workflows raises novel legal and implementation challenges because it increasingly influences care without fitting neatly into existing regulatory categories. Many AI tools are neither regulated medical devices nor purely administrative systems, yet they shape how clinicians interpret information, prioritize tasks, and frame clinical decisions.

This creates a structural accountability gap. In practice, vendor contracts typically disclaim clinical liability, while clinicians and provider organizations remain fully responsible for care decisions. At the same time, existing legal and regulatory frameworks provide limited guidance on the appropriate scope, safeguards, and expectations for AI-supported workflows. The result is a system in which responsibility is concentrated on providers, but the rules governing acceptable AI use remain underdeveloped.

HHS is uniquely positioned to address this gap by clarifying accountability structures without shifting clinical responsibility away from licensed professionals. This includes reinforcing that clinicians remain responsible for care decisions, while also establishing clear federal expectations around permissible AI use, required human oversight, and baseline standards that AI tools must meet when used in clinical contexts. Such guidance would enable both providers and innovators to design within well-defined boundaries, reduce uncertainty, and support responsible innovation while maintaining patient safety and public trust.

4. What are the most promising AI evaluation methods for clinical care, and should HHS support them?

InStride Response:

Promising AI evaluation methods must extend beyond technical accuracy to include **human-centered and workflow-based measures**. In clinical care, safety and effectiveness depend not only on model performance, but on how AI affects clinician judgment, introduces or mitigates bias, and interacts with real-world workflows.

Effective evaluation should include:

- Pre-deployment testing in representative clinical settings
- Post-deployment monitoring for drift and unintended consequences
- Assessment of impact on care quality, safety, and patient experience

HHS support for applied evaluation frameworks, through grants, cooperative agreements, or public-private partnerships, would help standardize responsible AI deployment and accelerate learning across the healthcare system.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

InStride Response:

HHS can best support innovative and effective AI use by encouraging **use-case-specific, industry-driven validation frameworks** rather than one-size-fits-all certification requirements. AI used in clinical care varies widely in risk and impact, from administrative support to decision preparation, and validation approaches should reflect that variation.

Rather than focusing exclusively on model-level certification, HHS should support frameworks that assess:

- intended use and scope,
- required level of human oversight,
- integration into clinical workflows,
- and ongoing monitoring after deployment.

Supporting industry-led validation efforts, informed by providers and grounded in real-world care delivery, would promote safety and trust without slowing innovation. This approach would also allow standards to evolve alongside technology, rather than hard-coding requirements that quickly become outdated.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations, and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

InStride Response:

AI has met or exceeded expectations in areas that **support clinicians and enhance decision-making**, including administrative efficiency, documentation support, information synthesis, workflow coordination, and clinical decision support. In certain well-defined use cases, such as matching patients to appropriate levels of care or identifying clinical fit, AI tools can perform at or above human benchmarks. When designed appropriately, these applications reduce cognitive and administrative burden and allow clinicians to **operate at the top of their licenses**, spending more time on clinical judgment, patient engagement, and care delivery rather than clerical tasks.

Challenges arise not from AI's analytical capability, but from **how decision-support outputs are framed, scoped, and integrated into clinical workflows**. AI tools can introduce risk when outputs are presented without appropriate context, limitations, or uncertainty signals, or when they are applied beyond the conditions under which they were validated. In complex and nuanced care scenarios such as mental health, where therapeutic relationships, patient history, and developmental context are central, overgeneralization or overconfidence in AI outputs can unintentionally influence clinical judgment.

The most promising AI tools are those that:

- operate within clearly defined, bounded workflows aligned to validated use cases,
- make uncertainty, limitations, and confidence levels explicit, and
- are designed to be reviewed, contextualized, and integrated by clinicians as part of decision-making, rather than used in isolation.

AI tools built with these principles have the greatest potential to improve outcomes, enhance care quality, and reduce costs while preserving clinical autonomy, patient safety, and trust.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

InStride Response:

AI adoption decisions in healthcare are influenced by a combination of clinical leadership, compliance and risk management, IT and security teams, and executive leadership. Successful adoption requires alignment across these groups, as each brings a distinct perspective on patient safety, operational feasibility, regulatory risk, and organizational priorities.

Primary administrative hurdles include:

- uncertainty around liability and accountability when AI supports clinical workflows,
- challenges integrating AI into existing clinical workflows in ways that support clinical judgment and care delivery without increasing cognitive burden or disrupting patient care, and
- lack of clear regulatory guidance that leadership teams can rely on when making adoption decisions across multiple jurisdictions.

Clear federal expectations around permissible scope, required oversight, and accountability for AI-supported care would materially reduce internal friction. Such guidance would allow organizations to evaluate AI tools based on clinical and operational value, rather than perceived regulatory or legal risk, and would support more confident, responsible adoption.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

InStride Response:

Enhanced interoperability would most effectively accelerate AI development when it enables **reliable, well-governed access to clinically meaningful data**, rather than simply increasing data volume. For care delivery organizations, the greatest value lies in interoperable access to structured clinical data, longitudinal patient histories, outcomes data, and operational signals that reflect real-world care processes.

Standardization that preserves **data provenance, context, and auditability** is particularly important for AI used in clinical workflows. Without clarity on how data were generated, transformed, and applied, AI outputs can be difficult to evaluate, validate, or trust. Interoperability efforts that include benchmarking tools and shared evaluation standards would further support responsible AI development by enabling comparison across populations, settings, and time.

HHS can play a critical role by advancing interoperability frameworks that support safe, transparent, and accountable AI use, ensuring that data exchange accelerates innovation while maintaining patient trust and clinical integrity.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

InStride Response:

Patients and caregivers want AI to address practical, day-to-day challenges in healthcare, including difficulty accessing care, long wait times, administrative complexity, and fragmented care experiences. AI that helps clinicians spend more time with patients and less time on administrative tasks aligns closely with these expectations.

At the same time, patients and caregivers express concerns about privacy, accuracy, transparency, and the risk that AI could replace human judgment or weaken therapeutic relationships. These concerns are especially pronounced in mental health care for children and adolescents, where trust, developmental context, and continuity of care are central to effective treatment.

Responsible AI adoption must therefore prioritize clear communication about how AI is used, strong human oversight, and safeguards that reinforce, rather than diminish the human elements of care that patients and families value most.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

- a. Are there published findings about the impact of adopted AI tools and their use in clinical care?**
- b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?**

InStride Response:

HHS should prioritize **applied, real-world research** that examines how AI performs when embedded in actual care delivery environments. In particular, research is needed on the impact of AI-supported workflows on clinical decision-making, patient outcomes, clinician workload, and equity over time.

Mental health and pediatric populations warrant focused attention, as risks, benefits, and ethical considerations differ meaningfully from adult medical care. Longitudinal studies that assess safety, bias, and durability of AI performance are essential to building trust and informing responsible scaling.

While the existing literature typically focuses on technical performance or theoretical cost savings, greater emphasis is needed on implementation science in terms of how AI tools change care processes, redistribute work, and affect quality and access in practice. Public-private partnerships and pragmatic trials embedded in care settings would help translate innovation into measurable clinical benefit.